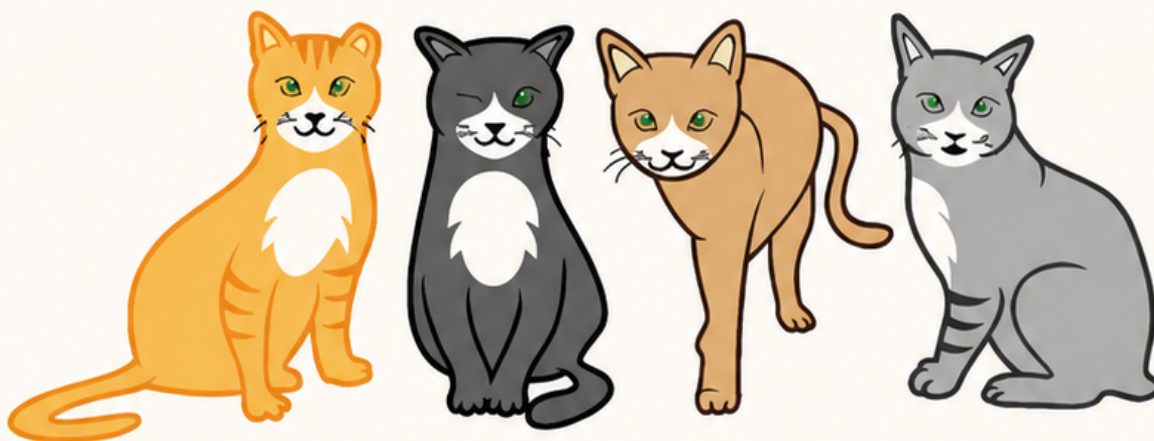




MEOWSQUAD



VOLUNTEER HANDBOOK



A volunteer-run rescue dedicated to helping
vulnerable cats throughout New York City through
rescue, foster placement, medical care,
rehabilitation, TNR efforts, and adoption.

RESCUE ♥ REHABILITATE ♥ REHOME

Together, we change lives. ♥

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1. Purpose, Scope, and SOP Updates

MeowSquad is a volunteer-run rescue dedicated to helping vulnerable cats throughout New York City through rescue, foster placement, medical care, rehabilitation, TNR efforts, and adoption. Because many cats entering our care are medically fragile, under-socialized, injured, orphaned, feral-leaning, or recovering from illness, consistency in care is essential.

These standard operating procedures create a shared baseline for daily care, cleaning, handling, documentation, foster support, volunteer communication, and emergency response. The purpose is to reduce confusion, improve animal welfare, protect volunteers, and keep MeowSquad organized and consistent.

Living document

- This handbook should be reviewed periodically and updated when MeowSquad identifies safer, clearer, or more effective procedures.
- Substantive changes should be approved by Stephanie Castro, Kathryn Rella, or designated leadership before being distributed.
- Volunteers and fosters should always follow the most current version provided by MeowSquad.

2. Volunteer Expectations and Chain of Communication

- Treat all animals, volunteers, adopters, fosters, veterinary staff, neighbors, donors, and members of the public respectfully.
- Follow instructions from Stephanie Castro, Kathryn Rella, and designated shift or program coordinators.
- Do not remove animals, medications, supplies, records, carriers, traps, or equipment without authorization.
- Immediately report injuries, illness, aggression, escape risks, bite or scratch incidents, unsafe conditions, missing supplies, or equipment concerns.
- Ask questions whenever unsure how to handle a cat, clean an area, administer care, or document a concern.
- Maintain confidentiality regarding sensitive rescue matters, medical cases, adopter information, internal conflicts, donor information, and private addresses.

Communication expectations

- Routine questions should go to the appropriate shift coordinator or program lead.
- Medical concerns, medication issues, emergencies, and contagious disease concerns should be escalated immediately to Stephanie or Kathryn.
- Volunteers should not make independent medical, adoption, intake, foster, or euthanasia decisions without leadership approval.
- If a volunteer feels unsafe or uncomfortable performing a task, they should stop and request help.

3. Required Records and Documentation

Accurate records help protect the cats, the rescue, volunteers, fosters, and adopters. Records should be maintained whenever possible for intake, medical care, vaccines, medications, surgery, foster placement, adoption, incidents, bite/scratch events, and return-to-owner situations.

- Each cat should have an identifying name or number, description, estimated age, sex if known, intake date, intake source, and location.
- Medical records should include exam notes, diagnoses, tests, vaccines, deworming, parasite treatment, medications, surgery, and veterinary recommendations.

5. Identification, Microchips, and Return-to-Owner Procedures

- All incoming cats should be scanned for a microchip as soon as safely possible.
- If a microchip is found, the chip number, registry information, and attempted contacts should be documented.
- If an owner or prior caretaker is located, leadership should determine the appropriate next step based on safety, legality, animal welfare, and the circumstances of intake.
- Owners claiming a cat should be asked for proof when possible, such as photos over time, veterinary records, microchip registration, adoption paperwork, or other identifying information.
- Return-to-owner decisions should be documented, including date, person released to, and any notes regarding condition or recommendations.
- If an animal appears abandoned, neglected, abused, or unsafe to return, leadership should be notified before any release decision is made.

6. Daily Animal Observation and Care

- Every cat should be visually observed daily for appetite, water intake, hydration, breathing, litterbox habits, stool quality, urination, mobility, energy level, stress, grooming, and general condition.
- Any cat appearing lethargic, dehydrated, congested, vomiting, unable to urinate, severely underweight, painful, collapsed, or in respiratory distress must be reported immediately.
- Cats recovering from illness, surgery, injury, anesthesia, or severe neglect may require individualized monitoring plans.
- Daily observations should be documented when a cat is ill, newly admitted, receiving medication, recovering from surgery, or being monitored for a known concern.
- Food refusal, repeated vomiting, diarrhea, blood in urine or stool, open-mouth breathing, severe limping, seizures, pale gums, or sudden behavior change should be escalated urgently.

7. Housing, Environment, Space, and Enrichment

- Cats should be housed in clean, safe areas that minimize stress, overcrowding, escape risk, injury, and disease transmission whenever feasible.
- Enclosures should be monitored daily for cleanliness, damage, sharp edges, loose doors, broken latches, unsafe bedding, exposed cords, or other hazards.
- Cats should have access to appropriate bedding, litterboxes, food, water, hiding spaces, and enrichment.
- Kittens, nursing mothers, medical cases, feral cats, aggressive or fearful cats, and contagious cats may require specialized housing arrangements.
- Lighting should generally follow normal day and night cycles whenever possible.
- Noise, unnecessary handling, chaotic movement, overcrowding, strong odors, and stressful introductions should be minimized.

Enrichment and stress reduction

- Provide gentle social interaction, quiet companionship, petting, brushing, treats, play, wand toys, scratchers, hiding spaces, and routine when tolerated.
- Do not force affection or interaction with fearful, overstimulated, painful, or feral cats.
- Allow shy cats to approach at their own pace whenever possible.
- Positive interaction and environmental enrichment help reduce stress, improve welfare, and support adoptability.

8. Feeding, Nutrition, Food Prep, and Dishes

- Fresh water must be available at all times unless otherwise directed by a veterinarian.
- Adult cats generally receive approximately one 5.5 ounce can of wet food daily, often divided into multiple feedings, along with dry food access unless otherwise directed.
- Kittens need significantly more calories and generally receive wet food three to four times daily with dry kitten food available throughout the day.
- Kittens under approximately two months of age should be fed at minimum three times daily and should never go prolonged periods without food.
- Underweight, pregnant, nursing, senior, diabetic, renal, GI, urinary, or medically fragile cats may require individualized feeding schedules or diets.
- Food intake should be monitored daily. Refusal to eat, vomiting, diarrhea, or sudden appetite change should be reported.
- Perishable food should be refrigerated appropriately and discarded if spoiled or left out too long.
- Food and water bowls should be cleaned and sanitized routinely and positioned so cats can safely access them.

Food preparation areas

- Food prep surfaces should be cleaned and disinfected regularly.
- Opened cans should be covered and refrigerated.
- Sick or isolation food items should not be mixed with general population supplies.
- Scoops, utensils, syringes, bowls, and measuring tools should be washed and disinfected between uses as appropriate.
- Hands should be washed or sanitized before and after food preparation, especially when moving between sick and healthy cats.

9. Sanitation, Disinfection, Laundry, and Litterboxes

- Approved disinfectants such as Rescue or other designated animal-safe disinfectants should be used according to manufacturer dilution instructions and contact times.
- Cleaning should generally move from healthiest animals to sickest or quarantined animals to reduce disease transmission.
- When spraying disinfectants near enclosures, cats should be temporarily moved or safely separated whenever possible.
- Separate cleaning supplies should be maintained for isolation, quarantine, holding, adoption, and general population areas whenever feasible.
- Food bowls, water bowls, litterboxes, exam tables, crates, carriers, traps, and common-contact surfaces should be cleaned and disinfected routinely.
- Items heavily contaminated with feces, urine, vomit, blood, discharge, or infectious material should be cleaned immediately.
- Enclosures should receive daily spot cleaning and regular full disinfecting.
- Enclosures should always be cleaned and disinfected before a new cat enters the space.
- Litterboxes should be scooped routinely and fully emptied and disinfected on a scheduled basis.
- Soiled bedding should be removed promptly and laundered thoroughly using detergent before reuse.
- Outdoor or storage areas should remain free from excessive clutter, waste buildup, fecal contamination, standing water, and pest attractants.

PPE and hand hygiene

- Gloves, gowns, towels, and other protective equipment should be used appropriately during cleaning, isolation care, medical care, and handling of bodily fluids.
- Hands should be washed or sanitized frequently, especially after handling cats, litterboxes, medications, bodily fluids, garbage, or contaminated supplies.
- PPE used in isolation areas should not be reused in healthy population areas.

10. Isolation, Quarantine, and Outbreak Management

- Cats showing signs of URI, ringworm, panleukopenia, diarrhea, vomiting, severe flea burden, unexplained wounds, or other potentially contagious illness should be separated from healthy cats whenever possible.
- Dedicated PPE, litterboxes, bedding, cleaning supplies, and feeding supplies should remain within isolation areas whenever feasible.
- Volunteers should minimize unnecessary traffic between quarantine/isolation and healthy population areas.
- Exposed cats should be monitored closely for symptoms and documented.
- Stephanie or Kathryn should be notified immediately regarding suspected contagious disease outbreaks.
- During outbreaks, leadership may implement enhanced cleaning, movement restrictions, isolation protocols, intake pauses, foster holds, adoption holds, or veterinary testing.

Disease warning signs

- URI: sneezing, nasal discharge, eye discharge, congestion, fever, reduced appetite, lethargy.
- Ringworm: circular hair loss, scaling, crusting, broken hairs, lesions, especially on ears, face, paws, or tail.
- Panleukopenia concern: vomiting, diarrhea, severe lethargy, anorexia, sudden decline, fever, or known exposure.
- Parasites: fleas, flea dirt, worms, diarrhea, weight loss, bloating, itching, or visible external parasites.

11. Safe Animal Handling and Restraint

- All handling should prioritize minimizing fear, stress, pain, and risk of injury to both cats and volunteers.
- Use calm voices, slow movements, towels, humane restraint techniques, traps, carriers, squeeze cages, nets, or other approved handling equipment when appropriate.
- Fearful, feral, painful, injured, or fractious cats should never be forced into unnecessary interaction.
- Physical force must never be used as punishment.
- Handling should prevent escape while minimizing distress.
- Carriers, traps, nets, squeeze cages, and other handling equipment should be maintained in safe working condition.
- Cats should never be restrained using control poles.
- If a cat becomes highly fearful, overstimulated, defensive, or unsafe to handle, stop and seek help rather than escalating the situation.

Special restraint situations

- Medical treatment, vaccines, nail trims, wound care, eye medication, and oral medications should be performed only by trained volunteers or under direction.
- Towel wraps, carrier dosing, trap dividers, and squeeze cages may be safer than direct handling for fractious cats.
- No volunteer should place themselves at unnecessary risk to complete a non-emergency task.

12. Pregnant, Nursing, Neonatal, and Young Kitten Care

- Pregnant and nursing cats should be housed in quiet, clean, low-stress areas with food, water, litter, bedding, and nesting space.
- Nursing mothers should receive adequate calories and should be monitored for appetite, hydration, milk production, discharge, pain, fever, mastitis, neglect of kittens, or distress.
- Neonatal kittens require close monitoring for warmth, hydration, weight gain, nursing, elimination, and signs of fading kitten syndrome.
- Young kittens should be handled gently, kept warm, and fed frequently according to age and veterinary or experienced foster guidance.
- Kittens under approximately four months of age require extra caution because stress, fasting, diarrhea, parasites, and URI can become dangerous quickly.
- Any kitten that is cold, limp, not eating, crying continuously, losing weight, having diarrhea, or struggling to breathe should be escalated immediately.

13. Behavioral Assessment, Socialization, and Stress Reduction

- Volunteers should monitor feline body language daily for fear, stress, aggression, pain, illness, overstimulation, shutdown, or behavioral decline.
- Cats may display stress through hiding, flattened ears, dilated pupils, growling, swatting, avoidance, decreased appetite, inappropriate elimination, overgrooming, or changes in activity.
- Fearful, undersocialized, feral, or traumatized cats should be handled with patience and realistic expectations.
- Behavior changes should always be reported because sudden behavioral changes may indicate pain or medical illness.
- Socialization should be gradual and cat-led whenever possible.
- Working cat or barn cat placement may be appropriate for cats who are not suited for traditional indoor adoption.

14. Grooming and Physical Care

- Cats should be visually assessed regularly for matting, fecal contamination, overgrown nails, skin irritation, flea dirt, wounds, ear debris, eye discharge, nasal discharge, and other grooming-related concerns.
- Long-haired cats may require regular brushing to prevent painful matting.
- Cats with diarrhea, mobility issues, obesity, senior changes, neurologic issues, or medical conditions may require sanitary cleaning or sanitary shaves.
- Grooming procedures should always prioritize minimizing stress and avoiding injury.
- Stop grooming attempts if a cat becomes highly distressed or unsafe to handle.
- Brushes, combs, nail trimmers, clippers, and grooming tools should be cleaned and disinfected between animals whenever appropriate.

15. Bite, Scratch, and Incident Procedures

- All bites and scratches, even seemingly minor ones, should be taken seriously and cleaned immediately. Cat bites can appear small while driving bacteria deep beneath the skin.

- Any volunteer bitten or scratched should immediately wash the affected area thoroughly with warm water and soap for several minutes. Wounds should then be disinfected using an antiseptic whenever available.
- Bleeding wounds should be allowed to gently flush initially when appropriate, then covered with clean gauze or bandages as needed.
- Monitor wounds for redness, swelling, heat, discharge, throbbing pain, fever, limited movement, red streaking, or worsening discomfort. These signs may require prompt medical care.
- Special caution should be used with puncture wounds, hand injuries, facial injuries, injuries near joints or tendons, deep scratches, uncontrolled bleeding, and wounds involving unknown vaccination status.
- Any moderate or severe bite or scratch must immediately be brought to the attention of Stephanie Castro or Kathryn Rella.
- Volunteers may be instructed to seek urgent medical attention depending on the severity, tetanus status, infection risk, and animal involved.
- All bite and scratch incidents should be documented with date, animal involved, circumstances, first aid, and whether medical care was recommended or obtained.

16. Major Medical Decisions and Humane Euthanasia Decisions

MeowSquad does not make major medical or humane euthanasia decisions casually. Decisions should be based on animal welfare, veterinary guidance, prognosis, suffering, safety, quality of life, available treatment options, and available resources.

- Major decisions should involve Stephanie, Kathryn, the treating veterinarian when available, and any other appropriate leadership or medical advisors.
- The primary concern should be preventing suffering while also considering safety, prognosis, and realistic treatment options.
- Euthanasia should never be based solely on space.
- When euthanasia is medically or behaviorally necessary, the date, reason, veterinarian, and circumstances should be documented.
- Volunteers should not announce, argue, or publicly discuss sensitive euthanasia or major medical decisions without authorization.

17. General Medical Maintenance and Physical Exams

- Cats should receive medical assessment appropriate to their condition, age, intake source, and risk level.
- Physical checks may include weight, body condition, hydration, gums, eyes, ears, nose, mouth, coat, skin, wounds, breathing, mobility, abdomen, and temperature when trained personnel can safely obtain it.
- New concerns should be documented and reported.
- Cats on treatment should be monitored for response, side effects, appetite, hydration, stool, vomiting, energy, and comfort.
- Medication should only be given as directed by MeowSquad leadership, veterinary instruction, or approved treatment protocols.
- Medication charts are reference tools only and must always be verified against medication concentration and veterinary guidance.

18. Parasite Control, Vaccinations, and Spay/Neuter Requirements

Parasite control

MeowSquad Volunteer and Operations Handbook

- Incoming cats should be assessed for fleas, flea dirt, ear mites, worms, diarrhea, external parasites, and skin concerns.
- Parasite treatment should be documented with product, dose, date, and person administering.
- Flea-infested cats should be handled carefully to prevent environmental contamination and anemia risk, especially in kittens.
- Deworming and repeat dosing should follow approved rescue or veterinary guidance.

Vaccinations

- Vaccination status should be documented whenever known.
- Vaccines should be administered according to veterinary guidance, age, medical status, and legal requirements.
- Sick, severely underweight, or unstable cats may require veterinary input before vaccination.
- Rabies vaccine documentation is important for bite incidents, adoptions, and legal compliance.

Spay and neuter

- Cats should be spayed or neutered before adoption whenever feasible and appropriate.
- Kittens, medical cases, pregnant cats, nursing mothers, and fragile cats may require individualized timing.
- NPO instructions, surgical appointments, transport plans, and post-operative monitoring should be clearly communicated.

19. Surgery, Anesthesia, NPO, Pain Management, and Emergency Care

Surgery and anesthesia

- Surgical candidates should be assessed for age, weight, health status, hydration, respiratory signs, pregnancy/nursing status, and current medications.
- NPO instructions should be followed carefully when required by the veterinary provider.
- Cats returning from anesthesia should be monitored for temperature, breathing, bleeding, pain, appetite, mobility, incision condition, and alertness.
- Post-operative instructions should be documented and followed.

Pain management

- Pain should be taken seriously and reported. Signs may include hiding, growling, reduced appetite, hunched posture, reluctance to move, rapid breathing, aggression, trembling, or changes in litterbox use.
- Only approved pain medications should be given. Human pain medications can be toxic and must never be administered unless specifically prescribed by a veterinarian.

Emergency veterinary care

- Respiratory distress, open-mouth breathing, urinary blockage symptoms, seizures, collapse, severe trauma, uncontrolled bleeding, inability to eat or drink, poisoning concern, severe dehydration, or extreme pain require immediate escalation.
- Emergency veterinary contact information should remain accessible.
- Leadership should be contacted as soon as possible, but life-threatening emergencies may require immediate transport based on established rescue direction.

20. Lost Animals, Transportation, Foster Program, and Volunteer Department

FEEDING GUIDELINES & NUTRITION STANDARDS

Proper nutrition is critical to maintaining the health, immune system, hydration, growth, and recovery of cats and kittens in MeowSquad's care. Volunteers and fosters should monitor appetite and food intake carefully and report sudden appetite changes, food refusal, vomiting, diarrhea, weight loss, or excessive hunger. General Adult Cat Guidelines: • Most healthy adult cats should receive approximately 1/4 cup of dry food twice daily unless otherwise directed. • Wet food may be offered daily to support hydration and appetite. • Underweight, senior, recovering, or medical cats may require additional calories or individualized feeding plans. • Fresh water should always be available. Kitten Feeding Guidelines: • Young kittens require more frequent feeding than adult cats. • Kittens under approximately 4 months old may require wet food multiple times per day in addition to dry food access. • Growth, weight gain, hydration, stool quality, and appetite should be monitored closely. • Neonatal kittens should only be fed approved kitten formula according to age and veterinary or experienced foster guidance. Medical & Special Diets: • Some cats may require prescription diets, limited ingredient diets, recovery diets, urinary diets, GI diets, kitten diets, or high-calorie support foods. • Volunteers should always check feeding instructions before offering food. • Cats with surgery restrictions, fasting instructions, or sedation requirements should not be fed unless specifically cleared. Feeding Safety: • Food bowls and water bowls should be cleaned regularly. • Uneaten wet food should not remain out for prolonged periods. • Any cat refusing food for more than 24 hours should be escalated promptly.

Lost animal procedures

- If a cat escapes or is discovered missing, notify leadership immediately.
- Secure doors, windows, carriers, traps, and nearby hiding areas.
- Do not chase a frightened cat unless immediate danger requires intervention.
- Document last known location, time, description, and circumstances.
- Use traps, cameras, feeding stations, flyers, microchip alerts, and local posts when appropriate and authorized.

Transportation

- All cats must be transported in secure carriers or humane traps appropriate for their size and temperament.
- Cats should never be transported loose inside vehicles.
- Animals should never be left unattended in unsafe temperatures.
- Carriers and traps should be labeled when appropriate and disinfected between uses.
- Transporters should confirm destination, appointment time, paperwork, medications, and special handling instructions before leaving.

Foster program

- Fosters are expected to follow MeowSquad sanitation, feeding, isolation, medication, and medical reporting protocols.
- Fosters should report illness, injury, appetite changes, vomiting, diarrhea, litterbox concerns, behavior changes, escape risks, and medication issues promptly.
- Fosters should not transfer, rehome, adopt out, medicate, or move cats without MeowSquad approval.
- Foster homes should be safe, clean, escape-conscious, and able to separate cats when needed.

Volunteer department

- Volunteers should be matched with tasks appropriate to their training, comfort, and reliability.
- New volunteers should receive orientation and should not be expected to perform advanced handling or medical tasks without training.
- Reliability is critical because missed shifts directly affect animal care.
- Volunteers are encouraged to communicate burnout, scheduling issues, safety concerns, or training needs.

21. Media Contacts, Facility Maintenance, Temperature, Ammonia, Sound, Light, Air, Pest Control

Media and public communications

- Volunteers should not speak to media, post sensitive internal information, or respond publicly on behalf of MeowSquad without authorization.
- Public statements should be handled by Stephanie or designated leadership.
- Photos, videos, medical details, adopter information, donor information, addresses, and internal conflicts should be shared only when appropriate and approved.

Outdoor and facility maintenance

- Outdoor areas, entryways, storage spaces, and animal areas should be kept free from excessive clutter, garbage, standing water, fecal buildup, unsafe materials, and pest attractants.
- Supplies should be stored safely and labeled when appropriate.

- Cleaning chemicals, medications, sharp objects, cords, and small hazardous items should be kept away from cats.

Temperature and ventilation

- Indoor housing should remain within safe temperature ranges for cats, with special attention to kittens, seniors, medical cases, and recovering animals.
- Extreme heat or cold should be addressed immediately through safe heating, cooling, ventilation, relocation, or veterinary care if needed.
- Airflow should reduce odor, humidity, and airborne irritants while avoiding unsafe drafts on fragile kittens or sick cats.

Ammonia, odor, and sound levels

- Strong ammonia odor may indicate inadequate ventilation, litterbox maintenance issues, overcrowding, or sanitation concerns and should be addressed promptly.
- Noise should be minimized whenever possible because loud, chaotic environments increase stress and may worsen illness or behavior.
- Normal light cycles should be maintained whenever feasible.

Pest and vermin control

- Food, litter, garbage, and supplies should be stored to reduce insects and rodents.
- Spills, crumbs, open food, and trash should be cleaned promptly.
- Pest control methods must be animal-safe and approved by leadership.
- Any evidence of pests should be reported and addressed quickly.

Clindamycin (25 mg/mL) Oral Liquid

Commonly used for abscesses, bite wounds, dental infections, osteomyelitis, and certain bacterial infections in cats.

Weight (lbs)	Approx Dose (mL)	Frequency
1	0.10	Every 12 hrs
2	0.20	Every 12 hrs
3	0.30	Every 12 hrs
4	0.40	Every 12 hrs
5	0.50	Every 12 hrs
6	0.60	Every 12 hrs
7	0.70	Every 12 hrs
8	0.80	Every 12 hrs
9	0.90	Every 12 hrs
10	1.00	Every 12 hrs
11	1.10	Every 12 hrs
12	1.20	Every 12 hrs
13	1.30	Every 12 hrs
14	1.40	Every 12 hrs
15	1.50	Every 12 hrs
16	1.60	Every 12 hrs
17	1.70	Every 12 hrs
18	1.80	Every 12 hrs
19	1.90	Every 12 hrs
20	2.00	Every 12 hrs

Important Notes: Clindamycin can cause gastrointestinal upset in some cats. Administer with food when possible.

Doxycycline

Broad-spectrum antibiotic commonly used for upper respiratory infections, Mycoplasma, Chlamydia, Bartonella, and some tick-borne diseases.

Weight (lbs)	Approx Dose	Frequency
3	1.36 mL	Every 12–24 hrs
4	1.82 mL	Every 12–24 hrs
5	2.27 mL	Every 12–24 hrs
6	2.72 mL	Every 12–24 hrs
7	3.18 mL	Every 12–24 hrs
8	3.63 mL	Every 12–24 hrs
9	4.09 mL	Every 12–24 hrs
10	4.54 mL	Every 12–24 hrs
11	4.99 mL	Every 12–24 hrs
12	5.45 mL	Every 12–24 hrs
13	5.9 mL	Every 12–24 hrs
14	6.36 mL	Every 12–24 hrs
15	6.81 mL	Every 12–24 hrs
16	7.26 mL	Every 12–24 hrs
17	7.72 mL	Every 12–24 hrs
18	8.17 mL	Every 12–24 hrs
19	8.63 mL	Every 12–24 hrs
20	9.08 mL	Every 12–24 hrs

Important Notes: Always follow oral dosing with food or water in cats to reduce risk of esophageal irritation.

Disclaimer: Educational veterinary-reference guide only. Always confirm medications, concentrations, and dosages with a licensed veterinarian before administering medication.

Terramycin Ophthalmic Ointment

Eye antibiotic ointment commonly used for conjunctivitis, eye infections, irritation, and URI-related eye discharge.

Application	Frequency
1/4-inch ribbon to affected eye(s)	2-4 times daily

Important Notes: Apply approximately a 1/4-inch ribbon to affected eye(s) 2-4 times daily.

Disclaimer: Educational veterinary-reference guide only. Always confirm medications, concentrations, and dosages with a licensed veterinarian before administering medication.

Azithromycin

Macrolide antibiotic commonly used for upper respiratory infections, Mycoplasma, Bartonella, and certain bacterial infections in cats.

Weight (lbs)	Approx Dose	Frequency
3	0.45 mL	Once daily
4	0.6 mL	Once daily
5	0.75 mL	Once daily
6	0.9 mL	Once daily
7	1.05 mL	Once daily
8	1.2 mL	Once daily
9	1.35 mL	Once daily
10	1.5 mL	Once daily
11	1.65 mL	Once daily
12	1.8 mL	Once daily
13	1.95 mL	Once daily
14	2.1 mL	Once daily
15	2.25 mL	Once daily
16	2.4 mL	Once daily
17	2.55 mL	Once daily
18	2.7 mL	Once daily
19	2.85 mL	Once daily
20	3.0 mL	Once daily

Important Notes: Azithromycin is often given once daily and may remain active in tissues after dosing. Always confirm concentration before calculating dose.

Disclaimer: Educational veterinary-reference guide only. Always confirm medications, concentrations, and dosages with a licensed veterinarian before administering medication.

CLAVAMOX DOSAGE CHART (CATS)

Clavamox (amoxicillin/clavulanate) is commonly prescribed for bacterial infections in cats including respiratory infections, wounds, abscesses, urinary infections, and dental infections. Always follow veterinary guidance when administering medications.

Weight (lbs)	Approximate Dose (mL)	Frequency
3 lbs	0.3 mL	Every 12 hrs
4 lbs	0.4 mL	Every 12 hrs
5 lbs	0.5 mL	Every 12 hrs
6 lbs	0.6 mL	Every 12 hrs
7 lbs	0.7 mL	Every 12 hrs
8 lbs	0.8 mL	Every 12 hrs
9 lbs	0.9 mL	Every 12 hrs
10 lbs	1.0 mL	Every 12 hrs
12 lbs	1.2 mL	Every 12 hrs
15 lbs	1.5 mL	Every 12 hrs
20 lbs	2.0 mL	Every 12 hrs

Important: Dosages may vary based on concentration and veterinary instructions. Medication should always be labeled with concentration, patient name, and dosing instructions.

Panacur (Fenbendazole)

Deworming medication commonly used for Giardia, roundworms, hookworms, and other intestinal parasites.

Weight (lbs)	Approx Dose	Frequency
3	0.6 mL	Once daily for 3–5 days
4	0.8 mL	Once daily for 3–5 days
5	1.0 mL	Once daily for 3–5 days
6	1.2 mL	Once daily for 3–5 days
7	1.4 mL	Once daily for 3–5 days
8	1.6 mL	Once daily for 3–5 days
9	1.8 mL	Once daily for 3–5 days
10	2.0 mL	Once daily for 3–5 days
11	2.2 mL	Once daily for 3–5 days
12	2.4 mL	Once daily for 3–5 days
13	2.6 mL	Once daily for 3–5 days
14	2.8 mL	Once daily for 3–5 days
15	3.0 mL	Once daily for 3–5 days
16	3.2 mL	Once daily for 3–5 days
17	3.4 mL	Once daily for 3–5 days
18	3.6 mL	Once daily for 3–5 days
19	3.8 mL	Once daily for 3–5 days
20	4.0 mL	Once daily for 3–5 days

Important Notes: Frequently used as a multi-day deworming protocol.

Disclaimer: Educational veterinary-reference guide only. Always confirm medications, concentrations, and dosages with a licensed veterinarian before administering medication.

Pyrantel Pamoate

Deworming medication commonly used for roundworms and hookworms in cats and kittens.

Weight (lbs)	Approx Dose	Frequency
3	0.3 mL	Single dose; repeat as directed
4	0.4 mL	Single dose; repeat as directed
5	0.5 mL	Single dose; repeat as directed
6	0.6 mL	Single dose; repeat as directed
7	0.7 mL	Single dose; repeat as directed
8	0.8 mL	Single dose; repeat as directed
9	0.9 mL	Single dose; repeat as directed
10	1.0 mL	Single dose; repeat as directed
11	1.1 mL	Single dose; repeat as directed
12	1.2 mL	Single dose; repeat as directed
13	1.3 mL	Single dose; repeat as directed
14	1.4 mL	Single dose; repeat as directed
15	1.5 mL	Single dose; repeat as directed
16	1.6 mL	Single dose; repeat as directed
17	1.7 mL	Single dose; repeat as directed
18	1.8 mL	Single dose; repeat as directed
19	1.9 mL	Single dose; repeat as directed
20	2.0 mL	Single dose; repeat as directed

Important Notes: Commonly used in kittens and often repeated in 2–3 weeks depending on parasite control protocols.

Disclaimer: Educational veterinary-reference guide only. Always confirm medications, concentrations, and dosages with a licensed veterinarian before administering medication.

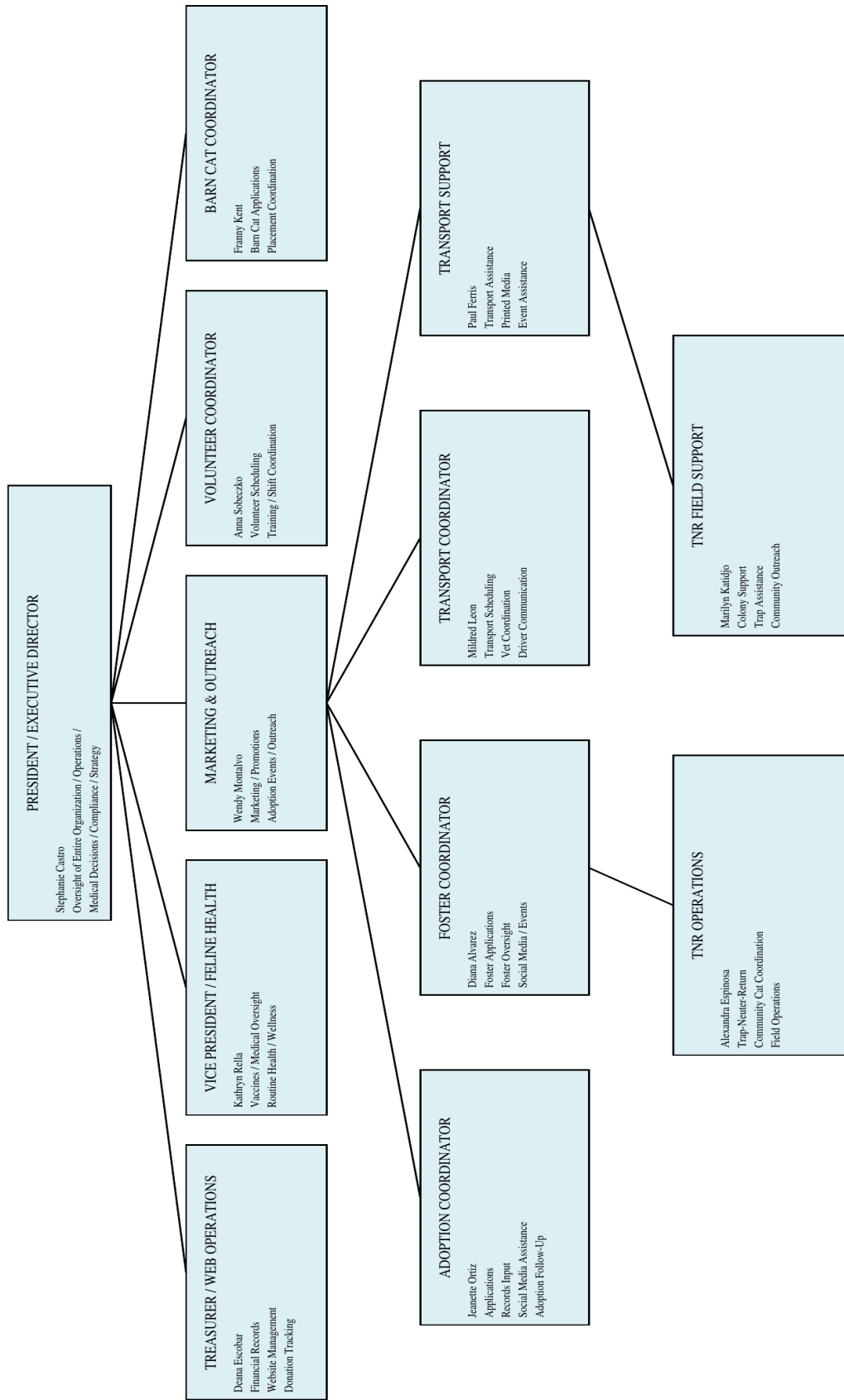
Preferred Veterinary Contacts

All veterinary visits, emergency visits, diagnostics, specialty consultations, or outside treatment must be authorized by MeowSquad leadership prior to scheduling whenever possible. Volunteers, fosters, and transporters should not independently seek veterinary care unless specifically directed or in a life-threatening emergency situation.

Veterinary Hospital	Purpose	Address	Phone
Guardian Veterinary Specialists	Emergency & Specialty Care	4 Hardscrabble Heights Brewster, NY 10509	(914) 704-3400
St. Mina Animal Clinic	General Veterinary Care	76-23 Jamaica Ave Woodhaven, NY 11421	(718) 296-0500
Wayne Hills Animal Hospital	General Veterinary Care	296 Church Ln Wayne, NJ 07470	(973) 633-7550

Guardian Veterinary Specialists should primarily be utilized for emergency stabilization, advanced imaging, specialty consultations, surgery, cardiology, internal medicine, critical care, or other specialty-level diagnostics and treatment.

MEOWSQUAD ORGANIZATIONAL FLOW CHART



VET APPROVED FELINE PROTOCOLS

Pursuant to NYS Article 26-C, the following feline-specific protocols have been developed for MeowSquad to support consistent nutrition, intake review, physical monitoring, emergency escalation, vaccination, parasite control, sanitation, quarantine, humane handling, and veterinary oversight for cats and kittens in MeowSquad care.

(a) Nutrition and Feeding

- (1) Fresh water shall be available at all times unless otherwise directed by a veterinarian.
- (2) Bottle babies and neonatal kittens shall be fed age-appropriate kitten formula on an age-appropriate schedule by trained caregivers or experienced fosters.
- (3) Kittens eating independently shall generally be offered kitten-appropriate wet food 3-4 times daily, with dry kitten food available when appropriate.
- (4) Adult cats shall generally receive wet food daily, often divided into multiple feedings, with dry food access unless otherwise directed.
- (5) Pregnant, nursing, senior, diabetic, renal, urinary, GI, underweight, obese, or medically fragile cats may require individualized feeding plans or prescription diets.
- (6) Food refusal, sudden appetite change, vomiting, diarrhea, or inability to eat shall be reported to MeowSquad leadership.

(b) Physical Exam / Intake Assessment

- (1) A MeowSquad representative shall perform an initial visual intake assessment as soon as safely possible.
- (2) Assessment may include weight, body condition, hydration, eyes, ears, nose, mouth, coat, skin, wounds, flea burden, respiratory effort, mobility, pregnancy/nursing status, litterbox concerns, and behavior.
- (3) A formal veterinary exam shall be obtained when illness, injury, pain, pregnancy complications, severe parasite burden, respiratory distress, neurologic symptoms, urinary concerns, or other significant abnormalities are present.
- (4) Daily informal observations shall be completed for cats in care, especially newly admitted cats, kittens, medical cases, post-operative cats, and cats in quarantine or isolation.

(c) Emergency Veterinary Care

- (1) MeowSquad shall provide or consult with emergency veterinary care when a cat develops open-mouth breathing, severe respiratory distress, collapse, seizures, suspected urinary blockage, severe trauma, uncontrolled bleeding, severe dehydration, toxin exposure, extreme pain, or rapid decline.
- (2) Kittens who are cold, limp, crying continuously, not eating, losing weight, having severe diarrhea, or showing signs of fading kitten syndrome shall be escalated immediately.
- (3) Volunteers and fosters must notify Stephanie Castro, Kathryn Rella, or designated leadership immediately when emergency symptoms are observed.

(d) Pain Management

- (1) Pain shall be taken seriously and referred for veterinary assessment when indicated.
- (2) Signs may include hiding, growling, hunching, reduced appetite, reluctance to move, rapid breathing, aggression, trembling, limping, vocalizing, or sudden litterbox changes.
- (3) No human pain medication shall be given to a cat unless specifically prescribed by a licensed veterinarian.

(e) Vaccinations

FVRCP and rabies vaccination protocols shall be administered according to veterinary guidance, age, medical condition, exposure risk, intake status, and applicable legal requirements. Vaccination status shall be documented whenever known.

Kitten FVRCP Schedule

Age	Recommended Vaccine
4-6 weeks	FVRCP may begin in high-risk rescue/shelter environments when indicated
6-8 weeks	FVRCP vaccine
Every 3-4 weeks thereafter	Repeat FVRCP booster
16-20 weeks	Final kitten FVRCP booster

Rabies Vaccination

Rabies vaccination should generally be administered at approximately 12-16 weeks of age according to veterinary guidance and local legal requirements. Proof of rabies vaccination should be maintained in the cat's medical record whenever available.

Adult Cat Booster Recommendations

Vaccine	General Recommendation
FVRCP	Boost approximately 1 year after kitten series, then according to veterinary guidance and risk assessment
Rabies	Administer according to vaccine type, veterinary guidance, and legal requirements

- Sick, severely underweight, unstable, pregnant, or medically fragile cats may require veterinary review before vaccination.
- Cats with unknown vaccine history should generally be treated as unvaccinated unless records can be confirmed.
- Vaccines should only be administered by trained individuals acting under approved rescue or veterinary protocol.

(f) Parasite Control

- (1) Incoming cats shall be assessed for fleas, flea dirt, ear mites, worms, diarrhea, skin irritation, and visible external parasites.
- (2) Pyrantel, fenbendazole/Panacur, flea treatment, ear mite treatment, or other parasite treatment may be administered according to approved MeowSquad protocol or veterinary direction.
- (3) All parasite treatments shall be documented with date, product, concentration when applicable, dose, cat name, and person administering.
- (4) Cats with severe flea burden, diarrhea, suspected Giardia/coccidia, ringworm suspicion, or other contagious concerns shall be isolated when feasible.

(g) Anesthesia, Surgery, and Spay/Neuter

- (1) Surgical candidates shall be assessed for age, weight, respiratory signs, hydration, pregnancy/nursing status, current medications, and overall health.
- (2) NPO instructions and post-operative instructions from the veterinary provider shall be followed.
- (3) Cats returning from surgery/anesthesia shall be monitored for temperature, bleeding, breathing, alertness, appetite, pain, mobility, and incision condition.
- (4) Spay/neuter shall be completed before adoption whenever feasible and medically appropriate.

(h) Humane Euthanasia

- (1) Humane euthanasia decisions shall be based on animal welfare, veterinary guidance, prognosis, suffering, safety, quality of life, and available treatment options.
- (2) Euthanasia shall not be based solely on space.
- (3) When euthanasia is medically or behaviorally necessary, the date, veterinarian, reason, and circumstances shall be documented.

(i) Quarantine, Isolation, and Infectious Disease Control

- (1) Cats showing URI signs, diarrhea, vomiting, ringworm suspicion, panleukopenia concern, unexplained wounds, severe parasite burden, or other contagious symptoms shall be separated from healthy cats whenever feasible.
- (2) Dedicated PPE, litterboxes, bedding, cleaning supplies, and feeding supplies should remain within isolation areas whenever possible.
- (3) Hands shall be washed or sanitized before and after handling animals, medications, litterboxes, bodily fluids, or contaminated supplies.
- (4) Urine, feces, vomit, blood, discharge, and heavily soiled areas shall be cleaned promptly and treated with approved disinfectant according to contact-time instructions.
- (5) During outbreak concerns, leadership may implement enhanced cleaning, movement restrictions, quarantine extensions, intake pauses, adoption holds, foster holds, or veterinary testing.

(j) Humane Handling and Restraint

- (1) Cats shall be handled in a manner that minimizes fear, stress, pain, and injury risk.
- (2) Towels, humane restraint, carriers, traps, trap dividers, squeeze cages, and other approved handling equipment may be used when appropriate.
- (3) Fearful, feral, painful, injured, fractious, or overstimulated cats shall not be forced into unnecessary interaction.
- (4) Physical punishment, excessive force, and unsafe restraint are prohibited.

Veterinarian Signature: _____

Printed Name / Clinic: _____

Date: _____

BITE / SCRATCH INCIDENT REPORT

This form must be completed for any bite, scratch, puncture wound, or animal-related injury involving a volunteer, foster, transporter, visitor, adopter, or other person while acting on behalf of MeowSquad.

Date / Time of Incident:	
Person Injured:	
Role: Volunteer / Foster / Transporter / Visitor / Other:	
Phone / Email:	
Cat Name / ID / Description:	
Cat Rabies Vaccine Status, if known:	
Location of Incident:	
Type of Injury: Bite / Scratch / Puncture / Other:	
Body Part Injured:	
Was skin broken? Yes / No	
Bleeding? Yes / No	
First aid performed:	
Was wound washed with soap and warm water? Yes / No	
Was antiseptic used? Yes / No	
Was medical attention recommended? Yes / No	
Was medical attention obtained? Yes / No / Unknown	
Leadership notified: Stephanie / Kathryn / Other:	
Witnesses:	

Describe exactly what happened, including handling activity, warning signs, restraint used, and any contributing circumstances:

Follow-up notes, wound monitoring concerns, quarantine/rabies notes, or additional action taken:

Signature of injured person: _____ Date: _____

Leadership review: _____ Date: _____

MEDICATION ADMINISTRATION LOG

Medication must only be administered as directed by MeowSquad leadership, veterinary instruction, or approved treatment protocol. Confirm medication name, concentration, dose, frequency, and animal identity before administering.

Date	Time	Cat Name/ID	Medication	Concentration	Dose	Route	Given By	Notes / Reaction

Medication storage / expiration notes: _____

Reviewed by: _____ Date: _____

TRAINING AND EDUCATION RESOURCES

- Fear Free Education Resources - <https://www.fearfree.com/education-resources/>
- Association of Animal Welfare Advancement - <https://learning.theaawa.org/>
- ASPCAPro New York State Companion Animal Care Standards Act Training - <https://www.aspcapro.org/new-york-state-companion-animal-care-standards-act-training>
- NYS Animal Protection Federation Training Resources - <https://nysapfedfund.org/training-resources-and-videos/>
- NYSAPF Foster-Based Rescue Standards Webinar - <https://nysapf.org/foster-based-rescues-and-the-standards-act-webinar/>
- Maddie's University Learning Catalog - <https://university.maddiesfund.org/>

PROTOCOL TEMPLATES

- NYSAPF Standard Operating Procedure Templates - <https://nysapfedfund.org/standard-operating-procedures-sops/>

ANIMAL HOUSING & FACILITY MONTHLY CHECKLIST

Monthly checklist: complete one column each week. Enter the date completed and initials of the person verifying completion.

Checklist Item	Week 1 Date	Initials	Week 2 Date	Initials	Week 3 Date	Initials	Week 4 Date	Initials
Housing areas free from overcrowding								
Separate housing for sick/quarantine animals								
Food and water accessible								
Litterboxes clean and accessible								
Safe bedding available								
No exposed wires/sharp hazards								
Proper ventilation maintained								
Safe temperature maintained								
Noise levels minimized								
Enclosures secure and escape-proof								

Month: _____

Reviewed by: _____

FOSTER HOME COMPLIANCE MONTHLY CHECKLIST

Monthly checklist: complete one column each week. Enter the date completed and initials of the person verifying completion.

Checklist Item	Week 1 Date	Initials	Week 2 Date	Initials	Week 3 Date	Initials	Week 4 Date	Initials
Quarantine area available if needed								
Windows/doors secure								
No toxic items accessible								
Food/water/litter available								
Emergency vet contacts available								
Medication instructions reviewed								
Resident animals vaccinated								
Foster understands reporting rules								
Foster agrees not to rehome animals								

Month: _____

Reviewed by: _____

TRANSPORT MONTHLY CHECKLIST

Monthly checklist: complete one column each week. Enter the date completed and initials of the person verifying completion.

Checklist Item	Week 1 Date	Initials	Week 2 Date	Initials	Week 3 Date	Initials	Week 4 Date	Initials
Carrier/trap secure								
Animal cannot escape								
Appointment confirmed								
Medical paperwork included								
Medication included if applicable								
Vehicle temperature safe								
Animal not left unattended								
Emergency contact available								
Handling instructions reviewed								
Carrier disinfected after use								

Month: _____

Reviewed by: _____

CLEANING & DISINFECTION MONTHLY CHECKLIST

Monthly checklist: complete one column each week. Enter the date completed and initials of the person verifying completion.

Checklist Item	Week 1 Date	Initials	Week 2 Date	Initials	Week 3 Date	Initials	Week 4 Date	Initials
Litterboxes scooped								
Litterboxes disinfected								
Food bowls cleaned								
Water bowls cleaned								
Laundry completed								
Floors disinfected								
Trash removed								
Carriers disinfected								
Disinfectant contact times followed								
Food prep surfaces disinfected								

Month: _____

Reviewed by: _____

MEOWSQUAD DAILY CAT ROOM OPERATIONS SOP

These procedures are intended to create consistent standards for feeding, cleaning, sanitation, monitoring, restocking, and safety throughout MeowSquad holding areas, foster overflow spaces, quarantine rooms, and general cat housing areas. All volunteers, fosters, and caregivers should follow these procedures whenever possible to help reduce disease transmission, improve animal welfare, maintain organization, and support NYS Article 26-C compliance standards.

Required Supplies

- Gloves or other appropriate PPE
- Food, water bowls, and feeding supplies
- Clean litter and litter pans
- Trash bags and laundry bins
- Disinfectant solutions approved by MeowSquad leadership
- Paper towels, towels, blankets, and linens
- Mop, bucket, broom, and dustpan
- Clean toys, enrichment items, and hide boxes
- Monitoring sheets and medication logs

General Safety Procedures

- Gloves should be worn whenever handling litterboxes, bodily fluids, cleaning chemicals, or cats with suspected illness.
- Gloves should be changed between quarantine areas, isolation spaces, and when heavily contaminated.
- Always review room notes, medical instructions, or special handling warnings before entering an area.
- Fearful, stressed, aggressive, painful, or feral cats should never be forcibly handled unless medically necessary.
- Any bite, scratch, escape, injury, or major concern must immediately be reported to leadership and documented.
- Wet floor signs should be used whenever floors are being mopped.
- If a cat escapes, nearby exits should immediately be secured and leadership notified.

Daily Feeding Procedures

- Each cat should receive appropriate food and fresh water according to age, health status, and veterinary guidance.
- Kittens, nursing mothers, seniors, underweight cats, and medical cases may require individualized feeding plans.
- Food and water bowls should be cleaned regularly and replaced when heavily soiled.
- Food intake should be monitored daily and poor appetite, vomiting, diarrhea, or refusal to eat should be reported.
- Cats with surgery restrictions, medical holds, fasting requirements, or sedation instructions should not be fed unless specifically cleared.

Occupied Room Cleaning Procedures

- Review all medical, quarantine, behavior, or feeding notes before beginning.
- Remove food bowls, water bowls, and litterboxes carefully to minimize stress.
- Litterboxes should be scooped or fully changed as appropriate based on contamination level.
- Heavily soiled litterboxes should be replaced and disinfected separately.
- Dirty linens, towels, and blankets should be replaced promptly.
- Hard surfaces should be disinfected using approved rescue disinfectant.
- Disinfectants must remain wet for the manufacturer-recommended contact time to properly disinfect.
- Isolation or illness areas may require extended disinfectant soak times.

- Glass, counters, shelves, carriers, cages, and high-contact surfaces should be cleaned regularly.
- Rooms should be securely latched before leaving the area.
- Monitoring sheets should be completed daily and serious medical concerns escalated immediately.

Complete Empty Room Sanitation

- All bowls, litterboxes, linens, toys, and removable items should be removed before deep cleaning.
- All surfaces should be disinfected thoroughly including walls, floors, doors, shelves, crates, and cat furniture.
- Isolation or outbreak rooms may require longer disinfectant contact times.
- Rooms should be allowed to dry before re-use whenever feasible.
- Fresh bedding, litterboxes, enrichment, and feeding supplies should be placed before admitting another cat.
- Clean and dirty supplies should remain separated whenever possible.

Daily Monitoring Requirements

- All cats should be visually observed daily for appetite, breathing, hydration, litterbox habits, behavior, injuries, and stress.
- Any cat appearing lethargic, congested, painful, dehydrated, unable to urinate, severely underweight, or in respiratory distress should be escalated immediately.
- Behavior changes should also be documented because they may indicate illness or stress.
- Monitoring logs should remain organized and accessible.

Restocking & End-of-Shift Procedures

- Dirty dishes, carriers, traps, litter pans, and cleaning tools should be sanitized before reuse.
- Dirty laundry should be placed in designated laundry areas.
- Trash should be removed regularly.
- Cleaning chemicals and medications should be stored safely away from animals.
- Food, litter, gloves, PPE, linens, and other supplies should be restocked before leaving the area.
- Floors should be swept and mopped using approved disinfectant dilution ratios.

Volunteer Training & SOP Acknowledgment

- Volunteers and fosters should receive orientation regarding sanitation, handling, medication safety, quarantine, and emergency reporting procedures.
- Questions regarding procedures should always be directed to Stephanie Castro, Kathryn Rella, or designated leadership.
- Failure to follow sanitation or safety procedures may place animals and people at risk and may result in removal from volunteer responsibilities.

MEOWSQUAD FEAR-FREE FELINE INTAKE & BEHAVIOR SOP

Entering rescue care is extremely stressful for many cats and kittens. MeowSquad aims to reduce fear, stress, disease exposure, and unnecessary handling whenever possible. Intake procedures should prioritize safety, calm handling, disease prevention, and allowing cats time to decompress.

Pre-Intake Preparation

- Prepare housing areas before the cat arrives whenever possible.
- Ensure clean litter, food, water, bedding, and hiding areas are available.
- Fearful cats should have access to hide boxes, covered carriers, or partial visual barriers.
- Quarantine or isolation areas should be prepared for sick or vulnerable cats.
- Kittens, neonates, nursing mothers, and medically fragile cats should be prioritized for rapid placement into foster care whenever feasible.

Fear-Free Intake Procedures

- Observe the cat's body language before handling.
- Allow cats to exit carriers voluntarily whenever possible.
- Never shake, bang, poke, or aggressively force a cat from a carrier.
- If a cat is extremely fearful or stressed, allow additional decompression time.
- Towels, traps, transfer cages, and humane handling techniques should be used whenever needed for safety.
- Volunteers should avoid loud voices, excessive eye contact, sudden movements, or crowding around fearful cats.

Escape Prevention Procedures

- All doors, windows, carriers, and enclosures should be secured before intake begins.
- If a cat escapes, nearby exits should immediately be secured.
- Volunteers should avoid chasing loose cats, as this often increases fear and injury risk.
- Leadership should be notified immediately if an animal escapes containment.
- Humane trapping or low-stress recovery plans should be implemented when necessary.

Feline Intake Medical Standards

- Adult cats should receive age-appropriate FVRCP vaccination unless medically contraindicated or verifiable records exist.
- Rabies vaccination should be administered according to veterinary guidance and NYS law.
- Kittens should begin FVRCP vaccination around 4–6 weeks of age and continue boosters approximately every 2–4 weeks until series completion.
- Parasite prevention and deworming should be administered according to veterinary direction and weight-based dosing.
- Flea infestations, URI symptoms, diarrhea, ringworm concerns, wounds, dehydration, respiratory distress, or neurologic symptoms should be escalated immediately.
- Microchips should be scanned at intake whenever possible.

Kitten & Neonatal Intake Considerations

- Young kittens should be monitored closely for dehydration, hypoglycemia, failure to thrive, diarrhea, URI symptoms, and temperature instability.
- Nursing mothers and neonatal kittens should be moved into foster care as quickly as feasible whenever possible.

- Underage or underweight kittens should not undergo sterilization procedures until medically appropriate.
- Weight tracking should occur routinely for neonatal and young kittens.

Behavior & Foster Monitoring

- Behavior observations should be documented routinely throughout foster care and holding.
- Volunteers and fosters should document appetite, litterbox habits, sociability, handling tolerance, stress, and behavioral concerns.
- Behavior notes can assist with placement decisions and identifying medical issues.
- Cats exhibiting severe stress, aggression, shutdown behavior, or litterbox avoidance should be reviewed by leadership.

Carrier & Transport Guidance

- Carriers should remain covered during stressful transport whenever appropriate.
- Cats should never be transported loose inside vehicles.
- Transporters should minimize loud noises and unnecessary handling.
- Fearful or fractious cats may require trap-based transport or additional safety measures.

FOSTER / BEHAVIOR PROGRESS REPORT

Return Date: _____

Foster Name: _____

Cat Name: _____

Length of Time in Foster: _____

How did the cat behave overall in the home environment?

How did the cat interact with other cats, dogs, adults, children, or visitors?

Describe litterbox habits and litter preferences.

Describe appetite, feeding habits, and preferred foods.

Did the cat display any medical or behavioral concerns?

How did the cat handle medication administration?

How did the cat react to handling and carriers?

What type of home would best suit this cat?

Additional notes or observations:

VOLUNTEER / FOSTER ACKNOWLEDGMENT

I acknowledge that I have received, reviewed, and understand the MeowSquad Volunteer & Foster Handbook. I understand that these policies and procedures are intended to help maintain the safety, health, and welfare of the animals in MeowSquad's care, as well as volunteers, fosters, visitors, and staff. I agree to follow MeowSquad protocols regarding animal handling, sanitation, medication administration, quarantine procedures, reporting of illness or injury, confidentiality, and general rescue operations. I understand that failure to follow established policies or conduct that places animals or people at risk may result in suspension or termination of volunteer/foster privileges.

Printed Name: _____

Signature: _____

Date: _____

Phone Number: _____

Email Address: _____

Emergency Contact: _____

MeowSquad Infectious Disease Prevention & Response Protocol

This protocol establishes the infectious disease prevention, monitoring, cleaning, quarantine, and outbreak response procedures utilized by MeowSquad in accordance with accepted shelter medicine guidelines and New York State Article 26-C standards. The purpose of this document is to reduce the spread of infectious disease, protect the health and welfare of animals in care, and ensure safe and sanitary conditions for volunteers, fosters, visitors, and staff.

1. Intake Examination Procedures

All incoming cats and kittens should receive an intake evaluation as soon as reasonably possible. Animals should be visually assessed for signs of illness including nasal discharge, sneezing, coughing, eye discharge, diarrhea, vomiting, lethargy, wounds, hair loss, parasites, dehydration, neurologic symptoms, or respiratory distress. Any animal displaying signs of infectious disease should be separated from the general population immediately. Basic intake records should include estimated age, sex, color, intake date, vaccination status if known, medical concerns, behavior notes, and source location when available.

2. Vaccination & Parasite Prevention

Core vaccinations should be administered in accordance with veterinary guidance whenever medically appropriate. Deworming and flea prevention should be initiated promptly upon intake unless medically contraindicated. Cats suspected of carrying parasites, ringworm, upper respiratory infections, or gastrointestinal illness should be monitored closely and housed separately when necessary.

3. Quarantine Procedures

New arrivals should ideally undergo a quarantine or observation period before being introduced into the general population. During this period, volunteers and fosters should monitor appetite, litter box usage, energy level, sneezing, coughing, vomiting, diarrhea, and overall demeanor. Whenever possible, healthy cats should be cared for before quarantined animals in order to reduce disease transmission.

4. Isolation Procedures

Cats with confirmed or suspected contagious illness should be housed separately from healthy animals whenever feasible. Dedicated litter boxes, bowls, bedding, cleaning supplies, and protective equipment should be used for isolation areas. Volunteers should wash hands thoroughly between handling animals and should avoid cross-contamination between healthy and sick populations.

5. Cleaning & Disinfection Protocols

All cages, carriers, litter boxes, bowls, bedding, floors, and frequently touched surfaces should be cleaned regularly using animal-safe disinfectants effective against common feline pathogens. Cleaning should proceed from healthy populations to quarantine and isolation populations whenever possible. Organic debris should be removed prior to disinfectant application. Shared supplies should be minimized.

6. Personal Protective Equipment & Hygiene

Volunteers and fosters should practice proper hand hygiene before and after handling animals. Gloves should be used when handling bodily fluids, heavily soiled materials, or animals with suspected contagious disease. Any volunteer experiencing illness that could compromise vulnerable animals should avoid direct contact with kittens, immunocompromised cats, or quarantine populations.

7. Common Infectious Diseases

Common infectious diseases encountered in rescue environments may include: • Upper Respiratory Infections (URI) • Panleukopenia • Ringworm • FeLV/FIV • Giardia and intestinal parasites • Fleas and mites • Conjunctivitis • Calicivirus and herpesvirus Animals displaying symptoms should be evaluated and monitored appropriately, with veterinary care sought when necessary.

8. Outbreak Response Procedures

If multiple animals begin exhibiting similar symptoms, MeowSquad may temporarily suspend intake, movement, or adoptions as necessary in order to contain potential outbreaks. Enhanced cleaning protocols, veterinary consultation, population separation, and medical monitoring should be implemented promptly during suspected outbreaks.

9. Foster Biosecurity Guidelines

Fosters should separate new or medically vulnerable animals from resident pets whenever possible. Food bowls, litter boxes, and bedding should not be shared between unrelated groups of animals. Fosters are expected to promptly report concerning symptoms including lethargy, refusal to eat, labored breathing, vomiting, diarrhea, or neurologic changes.

10. Recordkeeping Requirements

Accurate medical and observational records should be maintained whenever possible, including vaccinations, medications, surgeries, behavioral observations, illness concerns, and veterinary recommendations. Volunteers and fosters may be asked to maintain observation logs, socialization notes, feeding information, and medical updates for animals in their care.

11. Veterinary Oversight

MeowSquad works alongside licensed veterinary professionals for diagnosis, treatment recommendations, vaccinations, spay/neuter procedures, emergency care, and infectious disease management. Any severe or rapidly progressing illness should be brought to leadership attention immediately and veterinary care should be sought as appropriate.

This protocol may be updated periodically in accordance with evolving shelter medicine recommendations, operational needs, or New York State requirements.

MEOWSQUAD FOSTER CONTRACT

This Foster Agreement Contract ("Agreement") is entered into between **MeowSquad** and the undersigned foster caregiver ("Foster"). This Agreement applies to any and all animals fostered through MeowSquad now or in the future and outlines the responsibilities, expectations, safety procedures, liability provisions, veterinary requirements, and legally binding terms associated with fostering animals under the care of MeowSquad.

1. OWNERSHIP OF ANIMALS

All foster animals remain the sole legal property of MeowSquad at all times. MeowSquad reserves the right to reclaim, transfer, remove, or otherwise take possession of any foster animal at any time, with or without cause.

Initials

2. APPLICABILITY OF AGREEMENT

This Agreement applies to all animals fostered through MeowSquad during the Foster's relationship with the organization, including multiple foster placements over time.

Initials

3. FOSTER RESPONSIBILITIES

Foster agrees to provide safe, clean, humane, sanitary, and escape-conscious housing conditions; appropriate food and fresh water daily; clean litter boxes and bedding; daily monitoring; reasonable enrichment and socialization; and reasonable precautions to prevent illness, injury, escape, neglect, or mistreatment. Cats must remain inside at all times.

Initials

4. QUARANTINE, ISOLATION & DISEASE CONTROL

Foster acknowledges that newly rescued animals may require quarantine, decompression, or medical isolation. Foster agrees to follow all quarantine, sanitation, and separation instructions provided by MeowSquad.

Initials

5. DAILY MONITORING & WEIGHT REQUIREMENTS

Foster agrees to closely monitor foster kittens and medically fragile cats daily. Foster understands that kittens may be required to be weighed at least once daily using a gram scale provided or approved by MeowSquad and agrees to maintain accurate weight records.

Initials

6. RECORDKEEPING & OBSERVATION LOGS

Foster agrees to maintain reasonable records regarding the condition, behavior, appetite, weight, litter box habits, medications, socialization progress, cleanliness, medical concerns, and general wellbeing of foster animals whenever requested by MeowSquad.

Initials

7. ADOPTION PHOTOS, INFORMATION & MARKETING

Foster agrees to reasonably assist MeowSquad with adoption efforts by providing updated photographs, videos, personality information, behavioral observations, socialization updates, food preferences, litter box habits, compatibility information, and other details that may assist in marketing, placement, or adoption of foster animals.

Initials

8. VACCINATIONS & ROUTINE MEDICAL CARE

Foster acknowledges that MeowSquad or its authorized veterinary providers will administer vaccinations and routine veterinary care unless otherwise instructed. Foster agrees to make reasonable efforts to transport foster animals to scheduled appointments or agreed meeting locations.

Initials

9. MEDICAL MONITORING & REPORTING

Foster agrees to immediately notify MeowSquad of any signs of illness, injury, lethargy, appetite changes, vomiting, diarrhea, respiratory symptoms, limping, wounds, behavioral concerns, escape risks, or emergencies.

Initials

10. VETERINARY CARE & AUTHORIZATION

Veterinary treatment, diagnostics, emergency visits, specialty consultations, euthanasia decisions, or outside care must be approved by MeowSquad in advance whenever reasonably possible.

Initials

11. VETERINARY REGISTRATION REQUIREMENTS

All foster animals must remain registered under MeowSquad at all veterinary clinics, hospitals, specialty facilities, microchip registries, pharmacies, boarding facilities, and related service providers. Foster shall not represent themselves as the legal owner of any foster animal, alter ownership or microchip records, change the animal's name in official records without approval, add themselves to veterinary accounts as owner or co-owner, or obtain unauthorized medical care or treatment under their personal account unless authorized by MeowSquad. Foster acknowledges that all medical records, ownership records, veterinary authority, and adoption decisions remain solely under MeowSquad, and any attempt to misrepresent ownership or alter records may result in immediate termination of the foster relationship and recovery of the foster animal(s) by MeowSquad.

Initials

12. MEDICATIONS & MEDICAL TREATMENT

Foster shall not administer prescription medications, over-the-counter medications, flea preventatives, dewormers, sedatives, supplements, or medical treatments unless specifically authorized by MeowSquad or an approved veterinarian.

Initials

13. ESCAPE PREVENTION & TRANSPORT

Cats shall never be transported loose inside a vehicle and must remain secured in an appropriate carrier during transport.

Initials

14. ADOPTION & TRANSFER RESTRICTIONS

All adoption and placement decisions remain solely under the authority of MeowSquad. Foster shall not independently adopt out, transfer, surrender, abandon, breed, sell, or rehome foster animals.

Initials

15. SOCIAL MEDIA, CONFIDENTIALITY & PUBLIC REPRESENTATION

Foster may share approved photographs and updates regarding foster animals but agrees not to publicly misrepresent MeowSquad or disclose confidential rescue matters, donor information, adopter information, veterinary information, or internal rescue operations.

Initials

16. FOSTER HOME EXPECTATIONS

Foster certifies that the foster environment is safe for animals and that household members are aware of and agree to fostering. Foster further certifies that landlord approval has been obtained if applicable.

Initials

17. RETURN OF FOSTER ANIMALS

MeowSquad may request the return of any foster animal at any time. Foster agrees to immediately return foster animals upon request.

Initials

18. LIABILITY RELEASE & ASSUMPTION OF RISK

Foster understands that animals may bite, scratch, escape, damage property, transmit disease, or behave unpredictably. Foster voluntarily assumes all risks associated with fostering, including risks to themselves, household members, visitors, personal pets, and property.

Initials

19. TERMINATION

MeowSquad reserves the right to terminate this Agreement immediately, with or without cause, and remove foster animals if policies or instructions are violated.

Initials

20. GOVERNING LAW

This Agreement shall be governed by and interpreted under the laws of the State of New York.

Initials

21. TRAVEL & TEMPORARY CARE REQUIREMENTS

If Foster plans to travel, leave town, become unavailable, or otherwise be unable to personally care for foster animals for any period of time, Foster agrees to notify MeowSquad in advance whenever reasonably possible. Foster agrees that any temporary care arrangements, pet sitters, transfers, boarding, or return of foster animals must be approved by MeowSquad.

Initials

22. ACKNOWLEDGMENT

By signing below, Foster acknowledges that they have read, understood, and agree to comply with all terms established by MeowSquad.

Foster Printed Name: _____

Signature: _____

Date: _____

Address: _____

Phone: _____

Email: _____

Emergency Contact: _____

Emergency Contact Phone: _____

MeowSquad Representative: _____

Title: _____

Signature: _____

Date: _____